



Procedure

Cannabis Based Products: Patient's Own Medicines

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation wide	All Medical, Nursing and Pharmacy staff

Introduction

Cannabis Based Products (CBP) are available in a variety of different formulations. Prescribing, dispensing, administration, and storage of all CBP must comply with SCGOPHCG, NMHS and Statewide policy.

This procedure standardises processes for all SCGOPHCG staff involved in managing, storing and administering CBP Patient's Own Medicines (POM).

This procedure should be read in conjunction with:

- [SCGOPHCG Medicines Management Policy and Procedures](#)
- [SCGH Patient's Own Medicines \(POM\) Procedure](#)
- OPH Patient's Own Medicines (POM) Procedure

Risk Statement

Non-compliance with this procedure will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☒
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

1.0 Definitions

Cannabis	Mixture of chemical compounds derived from the plant Cannabis sativa including tetrahydrocannabinol (THC) and cannabidiol (CBD).
Cannabis Based Products (CBP)	A range of pharmaceutical cannabis preparations intended for therapeutic use. CBP includes all formulations of dronabinol, nabilone, nabiximols, tetrahydrocannabinol (THC), cannabidiol (CBD), cannabis and other cannabinoids included in Schedule 8 of the Poisons Standard. Medicinal cannabis based products supplied in Australia must meet Therapeutic Goods Administration (TGA) manufacturing quality standard TGO93.

Schedule 4 Prescription Only Medicine	Medicines that can only be obtained and administered with a legal prescription.
Schedule 8 Controlled Drug	Drugs of addiction. The S8 classification is for substances that should be available for use but require restriction to reduce abuse, misuse and physical or psychological dependence.
Schedule 9 Prohibited Substance	Substances which may be abused or misused and the manufacture, possession, sale or use of which should be prohibited by law except when required for: - Medical or scientific research, or - Analytical, teaching or training purposes.
Smoking	The inhalation and exhalation of the smoke of burning tobacco, herbs or drugs. This includes any method of consuming by combustion of plant material, herbs or drugs using any utensil or apparatus including cigarettes, pipes or cigars.
Vaporisation device	A device commonly used to heat dry plant extract or concentrated oils without burning the product produce fine aerosols in the form of vapour for inhalation. Vaporisation devices include: • Vaporisation devices registered as a medical device in Australia or overseas. • Non-registered devices intended for personal medicinal use. • Electronic cigarettes or other vaping apparatus.

2.0 General management of CBPs

CBP intended for therapeutic use are subject to the same regulatory controls as other medicines, in accordance with their Poisons Schedule. Only CBPs registered with the Therapeutic Goods Administration (TGA) or imported via the Special Access Scheme (SAS) are to be stored or used within the hospital.

Any CBP not legally prescribed and dispensed, such as recreational cannabis, remains under Schedule 9 of the Poisons Schedule and should be treated as per any other Schedule 9 Prohibited Substance. Such substances are not included within the scope of this document. Refer to the below for further information:

- [NMHS Policy: Prohibited Items in Healthcare Facilities](#)
- [SCGOPHCG Procedure: Disposal of Prohibited Items into the Safe/Dropbox](#)

This procedure outlines management of Patient's Own Medicines. For initiation of CBP products, prescribing must be in accordance with the Statewide Medicines Formulary requirements. Any prescribing outside the SMF requires an Individual Patient Approval (IPA) prior to initiation of treatment.

Patients must supply their own CBP for continuing treatment whilst an inpatient.

3.0 Storage of Patient's Own Medicines

The schedule of a CBP is determined by the Poisons Standard (SUSMP), according to the preparation and content.

Check the current version [SUSMP](#) to confirm the schedule of a CBP that is brought into hospital. The Poison Schedule (S3, S4 or S8) of a TGA registered CBP can also be determined by reviewing the TGA approved product information via [AusDI](#), MIMs or [TGA Product Information database](#). Staff who are unable to determine the Poison Schedule of a CBP or other special handling requirements should contact the ward pharmacist or on call pharmacist for advice.

Once the Poisons Schedule of the CBP is determined, refer to [SCGH Patient's Own Medicines \(POM\) Procedure](#) or OPH Patient's Own Medicines (POM) Procedure for general management of CBP in accordance with the product's Poison Schedule.

Examples current as at November 2023:

- CBPs comprising cannabidiol (CBD) 98% or more of the total cannabinoid content of the preparation (i.e. contains 2% or less of other cannabinoids found in cannabis), are classified as Schedule 4 Prescription Only Medicines
 - o These medicines (e.g. Epidyolex®) should be stored as per general requirements for Patient's Own Medicines
- Nabiximols (e.g. Sativex®), nabilone, dronabinol and products containing more than 2% tetrahydrocannabinols (THC) as a percentage of the total cannabinoid content of the preparation are classified as Schedule 8 Controlled Drugs
 - o These medicines have additional handling and storage requirements. Follow guidance for storage of S8 and S4R POM

It is the responsibility of the patient to ensure they have an adequate supply for the duration of their hospital stay. CBPs remain the property of the patient and should be returned at the point of discharge.

4.0 Prescription requirements on WA Hospital Medication Chart

Before prescribing and administering a CBP for continuation of therapy, it must be established that the product was legally prescribed and supplied to the patient. CBPs obtained online or through an overseas supplier are not lawfully obtained and should not be administered to inpatients at SCGOPHCG. Interstate visitors possessing a lawfully obtained product (meeting the legislative requirements of their originating state) may be administered their CBP.

A medical prescriber does not have to be a CBP authorised prescriber to continue an established treatment for an inpatient. Medical staff are required to review the indication of the CBP and assess whether it is clinically appropriate for pre-existing therapy to continue in the context of the current hospital admission. Confirmation of product, dose and legality is confirmed as per standard processes for obtaining a best possible medication history, as well as assessment of the POM for suitability for use. Any decisions made regarding continuation should be documented in the medical record and the CBP must be prescribed on the WA Hospital Medication Chart for administration.

5.0 CBP with special handling and administration requirements

5.1 Schedule 8 oral liquid or nasal spray products

Requirements for storage of CBPs are determined by the Poison Schedule.

For Schedule 8 Controlled Drugs, refer to:

- [SCGH Patient's Own Medicines \(POM\) Procedure](#):
 - o Section 2.1.5 Patient's own S8 and S4R liquid medications
 - o Section 2.3 Administration of S8 and S4R POM during Admission
- OPH Patient's Own Medicines (POM) Procedure

While in general, the administration of S8 POM during admission is discouraged, due to the nature of CBP, if it is determined that continuation of therapy is appropriate, use POM.

Due to the potential for volume loss with viscous liquids, any oral liquids should not be measured upon receipt. Inventory checks of S8 and S4R liquid medications are not to involve transfer, decanting or physical measurement, as this can result in further measurement loss, reduced potency, altered shelf life and potential contamination.

When the product is received from the patient, two authorised staff, in the presence of the patient where clinically appropriate and logistically possible, place the CBP into a tamper evident bag. Where appropriate, ask the patient to countersign acknowledgement of item and balance/count being received by ward.

All liquid S8/S4R Patient's Own Medicines (POM) must be stored in tamper-evident POM bags, regardless of whether it is being used for administration or not.

If a dose is needed, complete the required documentation in the S8 register, measure out the dose (do not measure actual content of the bottle) and seal in a new tamper-evident POM bag. This process is to be repeated for every dose dispensed from the liquid S8/S4R POM. This reduces diversion risk during time periods between administered doses. See appendix 1 for example of how to record patient's own CBP oral liquid in the S8 drug register.

There may be certain circumstances (or certain patient populations) where it is necessary to facilitate the patient witnessing access to the POM. In this scenario, two authorised staff take the sealed bag and register to the patient's bedside, demonstrating to the patient that the bag has remained intact, is opened in their presence and resealed in a new bag, and entered into the S8 register as such.

5.2 Products administered via vapourisation or inhalation via smoking

CBP administered via vapourisation of dried plant extract or concentrated oils through a vapourisation device, or via inhalation via smoking are not permitted within SCGOPHCG grounds in accordance with:

- MP0158/21 Smoke Free Policy
- NMHS Smoke Free Policy
- SCGOPHCG Smoke Free Procedure and
- WA Health Guidelines for the implementation of the Smoke Free Policy.

Vapourisation devices include TGA registered medical devices or overseas registered devices, non-registered vapourisation devices for personal use and electronic cigarettes (e-cigarettes). Inhalation via smoking includes the combustion of dried plant extract via a combustion device such as a pipe or cigarette (also known as a 'joint').

As per the policies listed above, patients who bring in CBP POM for vaporisation or inhalation via smoking, are not permitted to be prescribed and administered continuing therapy as an inpatient, and POM should be documented, handled and stored in accordance with the site POM procedure.

There may be extenuating circumstances where continuation of CBP via vaporisation or inhalation is deemed clinically appropriate by the treating team, who may escalate to executive for an exemption. In such circumstances, the rationale and a tailored patient specific approach for management should be determined by the treating team in consultation with nursing and pharmacy staff, and clearly documented in the integrated progress notes.

6.0 Compliance, monitoring and evaluation

Storage and handling of CBP will be assessed for compliance with and MP 139/20 Medicines Handling Policy requirements, SCGH Patient's Own Medicines (POM) Procedure and OPH Patient's Own Medicines (POM) Procedure.

7.0 Related Documents

Legislation:

- [Medicines and Poisons Act 2014](#)
- [Medicines and Poisons Regulations 2016](#)
- [The Poisons Standard](#)
- [Therapeutic Goods \(Standard for Medicinal Cannabis\) \(TGO 93\) Order 2017](#)

Policy and Procedures:

- WA Health MP0139/20 Medicines Handling Policy
- WA Health Guideline on oral liquid Schedule 4 Restricted and Schedule 8 medicines
- WA Health Guideline on patients own medicines
- WA Health Risk based requirements for medicines handling
- WA Health MP0158/21 Smoke Free Policy
- WA Health Guidelines for the Implementation of the Smoke Free Policy
- [WA Health Best Practice Principles for Medication Review Guidance Document](#)
- WA Health MP0077/18 Statewide Medicines Formulary Policy
- [Government of WA, Department of Health, Cannabis based products](#)
- NMHS Smoke Free Policy
- NMHS Administration of S8 and S4R Medicines Policy and Procedure
- NMHS Destruction, Discard and Disposal of S8 and S4R Medicines Policy
- NMHS Management of Patients' Own Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy
- SCGOPHCG Medicines Management Policy and Procedures
- SCGOPHCG Smoke Free Procedure
- SCGH Patient's Own Medicines (POM) Procedure
- OPH Patient's Own Medicines (POM) Procedure

Other

- [TGA Medicinal cannabis: Access pathways and patient access data](#)

Authorisation & Version Control

Policy Sponsor	SCGOPHCG Director of Clinical Services				
Policy Contact	SCGOPHCG Medicines and Therapeutics Committee Executive Officer				
Date First Issued:	31/05/2024	Last Reviewed:	31/05/2024	Review Date:	31/05/2029
Version No.	1	Reference #:	MMP014		
Approved by:	SCGOPHCG Nursing Practice Guideline Subcommittee			Date:	14/05/2024
Endorsed by:	SCGOPHCG Medicines and Therapeutics Committee			Date:	10/06/2024
Standards Applicable	NSQHS Standards (Version 2):  				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix 1 – Register entry for recording CBP oral liquid S8 POM receipt, administration, daily check and return to patient on discharge

RECORD OF DRUGS: RECEIVED, ADMINISTERED OR SUPPLIED

DRUG (including strength & fr Jane Smith Balance: nil Brought forward from page: new entry

Date	Received		Identifying details	Administered or Supplied							
	Amount	Requisition number	Full name of patient or supplier	Time	Dose administered	Amount discarded	Amount issued from stock	Balance	Name of Prescriber	Administered, Supplied or Received By (sign& print name)	Witnessed by (sign & print name)
01/12/2023	50mL		Received from J Smith sealed in Bag #XXXX	0945				50mL		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
01/12/2023			Bag #XXX opened for drug administration – J Smith	2000	1mL		1mL	49mL		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
01/12/2023			Bottle sealed in new Bag #YYY	2000				49mL		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
01/12/2023			Checked and confirmed Bag #YYY intact	2200				Bag intact		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
02/12/2023			Sealed bag #YYY returned to J Smith	0800			49mL	NIL		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO

*** CLOSE PAGE e.g. by drawing a diagonal line through the remaining space on the page and cross out the patient's name on the index page ***